General Survey & Health History

NURA 330 Health Assessment | Module 2 Lecture Study Guide

Exam 1 content · reviewed for the final

Lecture roadmap: Expected vs. unexpected findings → Descriptive vs. evaluative language → What a general survey is and its components → The introduction (safety, identifiers, AIDET, trauma-informed care) → Therapeutic communication → Doorway scan for distress → Appearance, behavior, body structure, mobility → Vital signs and pain → Abuse, neglect, and trafficking → Health history: biographic data, reason for seeking care, HPI (OLDCARTS), pain (PQRST), SDOH and function, past history, meds and allergies, immunizations, mental health, family history, social history, ROS → Closing the interview

Exam 1 blueprint: Module 2 was worth **18–20 questions**, the second-largest section. Priorities on the study guide: the four general survey areas, normal vs. abnormal vital signs, objective documentation, the doorway scan, two identifiers, AIDET, abuse/trafficking response, OLDCARTS and PQRST, allergy vs. side effect, three-generation family history, ADLs vs. IADLs, ROS pertinent positives/negatives, and closing the interview.

How to read this guide: Everything without a tag comes from the lecture slides or the slide notes. Anything else is tagged: STUDY GUIDE emphasized on the exam study guide but not spelled out on the slides; ADDED information not in the slides; CLARIFIED a slide point explained further; CORRECTED an error or inconsistency in the slides. Every tag is also listed in the notes at the end.

Part 1: The General Survey

Foundations: How to Think and Document

Expected vs. Unexpected Findings

Expected	Unexpected
Consistent with the patient's usual baseline Anticipated for age, developmental stage, and clinical context May include normal individual variation	New, changed, or different from baseline Outside the anticipated range or pattern Requires validation and further assessment; urgency depends on the finding

Key idea: Unexpected findings are cues for follow-up, not diagnoses. "Expected" does not mean perfectly healthy, and "unexpected" does not automatically mean disease. Always compare with baseline, age, and context.

Descriptive vs. Evaluative Language

Document what you observe or hear before assigning meaning to it. Descriptive language states observable facts; evaluative language adds conclusions or judgments.

Evaluative (avoid)	Descriptive (chart this)
"Patient looks anxious."	"Patient wrings hands, shifts repeatedly in the chair, and speaks rapidly."
"Poor hygiene."	"Clothing visibly soiled; strong body odor noted."
"Patient is uncooperative."	"Patient declined the examination and stated, 'I do not want to continue.'"
"Patient looks sick."	"Patient is pale, diaphoretic, and breathing rapidly while leaning forward."

Observation vs. diagnosis: "Patient is leaning forward and speaking in short phrases" is an observation. "She has COPD" is a diagnosis. In health assessment we make evidence-based observations.

What Is a General Survey?

The **initial and ongoing holistic appraisal** of a client's baseline health status; the first, overall inspection of the patient.

Timing	**Begins at the first encounter** (from the knock on the door) and **continues throughout** the relationship
Purpose	Establishes a baseline impression; flags issues needing a focused assessment
Action	Gathers early cues through sight, hearing, smell, and interaction; observes appearance, behavior, mobility, and distress

Components of the General Survey

STUDY GUIDE **The four main areas: Physical appearance, Body structure, Mobility, and Behavior.** The slide also lists vital signs and measurements, pain, signs of distress, and experiences of violence as part of the survey.

Component	What you observe
Appearance	Age, skin color and integrity, facial features, hygiene, dress
Behavior	Facial expression, mood, affect, speech clarity, demeanor
Body structure	Posture, symmetry, stature, nutritional status
Mobility	Ease of movement, gait, balance, range of motion
Vitals and measurements	Height, weight, BMI, full vital signs
Pain	Verbal and nonverbal cues
Signs of distress	Immediate indicators of acute physical or emotional compromise needing urgent action
Experiences of violence	Physical or behavioral indicators of trauma, IPV, or trafficking

The Introduction

Infection Control and Patient Safety First

1. **Knock**, announce yourself, and ask permission to enter.
2. **Hand hygiene** immediately on entering.
3. **Verify two patient identifiers** (full name and date of birth) while checking the armband. Asking the patient to *state* them also lets you assess level of consciousness and orientation.

AIDET

Letter	What to do
A: Acknowledge	Greet by name, make eye contact (expectations vary by culture), acknowledge visitors
I: Introduce	Your name, your role as a nursing student, and your purpose
D: Duration	How long the assessment or conversation will take
E: Explanation	What you will do, step by step, so there are no surprises
T: Thank you	Thank the patient and address immediate comfort needs

Trauma-Informed Approach

- **Environmental safety:** position yourself so the patient has a **clear, unblocked line of sight to the exit** and never feels trapped.
- **Identity and respect:** ask how they prefer to be addressed, including pronouns.
- **Shared decision-making:** involve the patient and get **verbal consent before touching**.

Therapeutic Communication

Verbal	Nonverbal
Begin broadly, then focus Listen without unnecessary interruption Clarify, reflect, summarize Plain language; one question at a time Acknowledge feelings without judgment	Open, calm posture; attentive presence Sit at eye level when possible Comfortable gaze; do not force eye contact Attend to tone, pace, expression, silence Respect personal space and culture

When words and nonverbal cues do not match, **clarify rather than assume**.

Signs of Distress: The Doorway Scan

Ask yourself: **does this person look immediately unstable?** Check responsiveness, breathing, circulation/color, and distress before anything else.

Category	Look for
Respiratory	Labored breathing, accessory muscle use, speaking in short phrases, cyanosis; also tachypnea, wheezing, stridor, nasal flaring, retractions, tripod positioning
Pain	Guarding, grimacing, moaning, restlessness, severe reported pain
Neurologic	New confusion, altered level of consciousness, weakness, speech change
Emotional	Marked fear, panic, agitation, shutting down, inability to engage
Cardiovascular (slide notes)	Profuse diaphoresis, severe pallor, cyanosis, weakness or fainting

Priority rule: If the survey suggests instability, **stop and prioritize immediate assessment and safety**. The routine survey and history can wait.

Appearance, Behavior, Structure, and Mobility

General Appearance

Domain	Expected	Unexpected / follow up
Facial appearance	Symmetric; spontaneous movement that changes with conversation	Droop or asymmetry; **masklike** expression (seen with Parkinson's, some psychiatric conditions, medications); involuntary movements; swelling
Hygiene, grooming, dress	Generally clean and cared for; appropriate to context	Marked change from baseline; visibly soiled or ill-fitting clothing; clothing unsuitable for weather; unusual odor
Skin / diaphoresis	Color consistent with baseline; skin intact	Pallor, cyanosis or dusky color, diaphoresis, new lesions
Comfort / fatigue / frailty	Comfortable at rest; energy fits the situation	Visible distress; marked fatigue, weakness, or frailty

Do not infer socioeconomic status, housing, motivation, substance use, or culture from appearance. Hygiene varies with culture, occupation, resources, and function. **Describe the finding first.**

Behavior

	Observe	Expected	Assess further
Speech	Clarity, fluency, rate, volume, articulation	Clear, fluent, understandable; rate and volume fit context	Slurred or garbled speech; word-finding difficulty; unusually rapid or slow speech
Mood & affect	**Mood** = what the patient reports feeling. **Affect** = the emotion you observe	Affect has an appropriate range; mood and affect match	Flat or restricted affect; labile affect; marked mismatch or distress

	Observe	Expected	Assess further
Interaction	Engagement, organization, interaction with you	Engages and responds appropriately	Agitation or withdrawal; confused or disorganized responses; change from baseline

Memory line: You observe affect; you ask about mood. Process: Observe → Describe → Clarify → Assess further. Avoid labels like "hostile" or "difficult"; consider culture, language, hearing, anxiety, and cognition before interpreting pauses or eye contact.

Body Structure, Posture, and Mobility

- **Build and symmetry:** body habitus, proportion, bilateral symmetry, obvious deformity.
- **Posture:** upright, stooped, rigid, slumped, guarded, asymmetric.
- **Positioning:** comfortable? leaning forward? guarding? unable to lie flat? Unexpected positioning can be an **early cue of pain, weakness, or respiratory distress**.
- **Mobility:** watch during routine movement: gait, balance, coordination, rising/sitting/transfers, assistive devices.
  - **Expected:** smooth, coordinated movement; steady gait; controlled transfers.
  - **Unexpected:** staggering, shuffling, limping, dragging, guarding, weakness, marked hesitation, unsafe transfers.

Vital Signs and Pain

Vital signs add objective physiologic data; pain screening adds the patient's subjective experience.

Measure	Adult reference (slide)	Abnormal term
Temperature	36–38 °C (96.8–100.4 °F)	**Fever/pyrexia**: above the range ADDED
Pulse	60–100 bpm at rest	**Tachycardia** >100; **bradycardia** <60
Respirations	12–20/min at rest	**Tachypnea** >20; **bradypnea** <12
Blood pressure	Systolic <120 **and** diastolic <80 mmHg	Hypertension / hypotension

Measure	Adult reference (slide)	Abnormal term
SpO_2	95–100%	Hypoxemia below range
Pain	Self-report + functional impact	Assess characteristics, severity, effect on function

Nursing considerations: interpret values against the patient's baseline and context; verify technique, route, cuff size, and equipment; **repeat unexpected readings** and compare with prior values and trends; escalate concerning trends or signs of instability.

STUDY GUIDE ADDED **What abnormal vital signs mean (the study guide asks this directly):**

- **Tachycardia:** the heart is working harder or compensating. Think fever, pain, anxiety, dehydration or blood loss, hypoxemia, infection, or medications. Action: look for the cause, recheck, assess perfusion and other VS, report if new or symptomatic.
- **Tachypnea:** the body is trying to get more oxygen or blow off CO_2. Think hypoxemia, respiratory illness, fever, pain, anxiety, metabolic causes. Action: check SpO_2, work of breathing, and lung sounds; position upright; report.
- **Fever:** often infection or inflammation; raises heart rate, respiratory rate, and oxygen demand. Action: assess for source and other signs of infection; report per policy.
- **General rule:** one abnormal number matters less than the pattern. Recheck, compare to baseline, look at the whole patient, and escalate when values are worsening or the patient is symptomatic.

Safety: Abuse, Neglect, and Human Trafficking

Potential indicators	Interview approach	Nursing response
Injuries or a story that does not fit the explanation or developmental ability Unmet needs (food, hygiene, meds, medical care, shelter) Fear, hypervigilance, withdrawal, or a companion who **controls the conversation or privacy**	**Speak with the patient alone** when safe Explain confidentiality and reporting limits Direct, **nonjudgmental** questions; a **qualified interpreter** (never the partner, family member, employer, or companion) **Do not pressure** for disclosure	**Address urgent medical and safety needs first** Document objective findings and the patient's own words Follow agency policy and **mandatory reporting** laws (vary by state, age, setting) Involve social work, forensic/SANE, advocacy, CPS/APS

Never confront a suspected abuser or trafficker. Confrontation can increase danger to the patient and staff. Indicators are cues, not proof; do not rely on a single red flag or stereotype.

- **PEARR** (HEAL Trafficking): **P**rovide privacy, **E**ducate, **A**sk, **R**espect, **R**espond. A conversation guide, not a checklist.
- **SOAR** (HHS): **S**top, **O**bserve, **A**sk, **R**espond. Free training for health professionals.
- **USPSTF (2025):** screen women of reproductive age (including pregnant and postpartum) for IPV. Evidence is insufficient for universal elder-abuse screening, but nurses should still recognize indicators.
- **Sample neutral questions:** "Do you feel safe where you live?" "Has anyone hurt, threatened, frightened, or controlled you?" "Is anyone controlling your money, ID, phone, transportation, or access to care?"

Part 2: The Health History

The patient's story of current and past health, including the experiences, lifestyle, and circumstances that shaped it. **The order can change with the setting, and immediate safety always comes first.** If the patient is unstable, pause the history. Focused histories may shorten or skip sections.

Current Health	Health Background	Context & Patterns
Biographic data Reason for seeking care HPI (OLDCARTS) SDOH and function	Past illnesses and injuries Hospitalizations and surgeries Medications and allergies Immunizations and prevention	Emotional/mental health Family history Personal and social history Review of systems

Biographic Data and Reason for Seeking Care

- **Biographic data:** preferred name and pronouns; age/DOB; preferred language and interpreter needs; source of the history when relevant. **Do not infer identity from appearance**; ask respectfully.
- **Reason for seeking care** (chief concern/complaint): start open, e.g., "What brought you in today?" Record it **verbatim, in the patient's own words**, in quotation marks. Do not convert it to medical terms.

History of Present Illness: OLDCARTS

Start open-ended ("Tell me more about...") then use OLDCARTS to fill in the details. The order does not need to be rigid.

Letter	Ask
O: Onset	When did it begin? Sudden or gradual?
L: Location	Where is it? Does it radiate or move?
D: Duration	How long does it last? Constant or intermittent?
C: Characteristics	What does it feel like?
A: Aggravating / Alleviating	What makes it worse or better?

Letter	Ask
R: Related symptoms	What else happens with it? (Points to other systems to assess)
T: Treatment	What have you tried? Did it help?
S: Severity	How severe? **How does it affect function?** (not just a number)

Pain Assessment: PQRST

Letter	Ask
P: Pattern / precipitating factors	What caused your pain? What makes it better or worse?
Q: Quality	Sharp, dull, burning, throbbing?
R: Radiates	Does it spread? Where?
S: Severity	0–10 (0 = no pain, 10 = worst pain ever)
T: Time and treatment	When did it begin? Are you taking anything for it?

CLARIFIED Many textbooks expand P as "**Provocation/Palliation**" and R as "**Region/Radiation**." The questions are the same; use whichever wording your instructor uses.

SDOH and Function

- **Social and practical needs:** housing and neighborhood safety; food and utilities; transportation; finances, work, and education; access to care and medications; relationships, support, and safety.
- **Normalize the questions:** "I ask everyone about things that can make it easier or harder to stay healthy..."
- **Healthy People 2030 SDOH domains:** economic stability; education access and quality; health care access and quality; neighborhood and built environment; social and community context. A positive screen should lead to resources or referral.

	ADLs (Activities of Daily Living)	IADLs (Instrumental ADLs)
What	Basic self-care	More complex tasks needed to live independently

	ADLs (Activities of Daily Living)	IADLs (Instrumental ADLs)
Examples	**Bathing, dressing, toileting, eating**	**Managing medications, preparing meals, shopping, finances, transportation**

Also ask about usual mobility and activity level, and changes from baseline.

Health Background

Area	What to collect
Childhood illnesses	Significant illness, complications, lasting effects (not just a checkbox)
Chronic illnesses	Diagnosis, approximate date, current treatment, how well controlled, specialist care
Injuries / trauma	What happened, when, treatment, residual effects
Hospitalizations	Reason, approximate date, course, complications
Surgeries / procedures	Procedure, date, reason, complications or recovery issues
Other	**Blood transfusions**, reproductive/obstetric history, exposures when relevant

Dates matter. Approximate dates are still useful.

Medications and Allergies

Medications	Allergies
Prescription, **OTC**, and **vitamins/supplements/herbals** (list each separately) For each: **name, dose, route, frequency, indication** Recent changes, missed doses, cost or access barriers Ask what they *actually* take, not just what is prescribed	Medication, food, latex, other **What reaction occurred? How severe?** **Distinguish a true allergy from a side effect or intolerance**

Allergy vs. side effect: "Allergy: nausea" is likely an **intolerance/side effect**. Hives, angioedema, wheezing, or anaphylaxis is a **true allergic reaction** and changes risk significantly. **Medication reconciliation** means comparing sources (record, bottles, pharmacy, caregiver) and resolving discrepancies, not copying the EHR list.

Immunizations and Health Maintenance

- **Immunizations:** routine and risk-based; verify dates when possible.
- **Screening:** cancer, cardiovascular, metabolic, infectious disease, and other age/risk-based screens.
- **Routine care:** primary care, dental, vision.
- **Health promotion:** nutrition, activity, sleep, tobacco/substances, sexual health.

Context and Patterns

Emotional and Mental Health

Ask: "How have you been doing emotionally?" Explore mood, anxiety, stress, sleep, energy, coping, supports, past diagnoses/treatment, trauma when relevant, and safety (including self-harm) when indicated. Emotional health is **not inferred from facial expression alone**. If a safety concern emerges, **pause the history and follow the escalation process**.

Family History

- Focus on **biologic relatives** for inherited risk, while recognizing that "family" can be broader.
- For key relatives: current age and health; major illnesses and **age at diagnosis**; if deceased, age and cause of death.
- Patterns: **early** cardiovascular disease or stroke, diabetes, hypertension, cancer, genetic disorders, mental health or substance use.
- A **three-generation** family history (patient's generation, parents, grandparents) is useful when inherited risk matters. STUDY GUIDE
- Be sensitive to adoption and estrangement. Family history is a risk factor, not a guarantee of disease.

Personal and Social History

Health practices	Life context
Nutrition and eating patterns Physical activity Sleep Tobacco/nicotine Alcohol and other substances: **what, how much, how often, when last used** (not "Do you abuse...?")	Occupation and exposures Sexual and reproductive health as relevant Relationships and support Spiritual/cultural practices important to care Safety at home, work, relationships

Review of Systems (ROS)

Focused questions, system by system, to find symptoms **not already captured in the HPI**: general, skin, HEENT, respiratory, cardiovascular, GI, GU/reproductive, musculoskeletal, neurologic, endocrine, hematologic/lymphatic, psychiatric.

ROS is subjective. Document **pertinent positives** (symptoms the patient has that matter) and **pertinent negatives** (symptoms the patient denies that matter, e.g., "denies chest pain"). **Objective findings belong in the physical exam, not the ROS.** CORRECTED see note 1.

Closing the Interview

1. **Summarize:** reflect back key concerns and history (lets the patient correct you).
2. **Validate:** clarify inconsistencies and missing details.
3. **Invite:** "Is there anything important we have not talked about yet?"
4. **Prioritize:** identify concerns needing focused assessment, intervention, or escalation.
5. **Document:** subjective data clearly and objectively; chief concern in the patient's words; no judgmental labels; note the source of collateral information.

Practice Case

Patient says: "I've been getting short of breath when I walk to the mailbox. It started about a week ago."

- **Reason for seeking care:** document the quote exactly.
- **OLDCARTS examples:** "Is it getting worse?" (onset/course) "What makes it better?" (alleviating) "Do you have chest pain or swelling with it?" (related) "What can you do now that you couldn't before?" (severity/function).
- **Past history to explore:** heart and lung conditions, medications, smoking history, prior hospitalizations.
- **SDOH/function:** "Who helps you at home?" "Are you able to get your groceries?"
- **When to stop and reprioritize:** shortness of breath at rest, cannot speak in full sentences, altered consciousness, severe chest pain, or other instability.

Quick Review

Concept	Key points
General survey	Starts at first contact, continues throughout; 4 areas: appearance, body structure, mobility, behavior
Documentation	Descriptive, not evaluative; unexpected = cue, not diagnosis
Introduction	Knock, hand hygiene, 2 identifiers, AIDET, clear path to exit, consent before touch
Doorway scan	Respiratory, pain, neuro, emotional; unstable = stop and prioritize
Mood vs. affect	Ask about mood; observe affect

Concept	Key points
Adult VS	T 36–38 °C; HR 60–100; RR 12–20; BP <120/<80; SpO_2 95–100%
Abuse/trafficking	Interview alone; qualified interpreter; no pressure; urgent needs first; mandatory reporting; never confront
Chief concern	Verbatim, in quotes
OLDCARTS / PQRST	Symptom analysis / pain analysis
Meds & allergies	Rx, OTC, supplements separately; allergy (hives, anaphylaxis) vs. side effect (nausea)
Family history	Biologic relatives; 3 generations; age at diagnosis
ADLs vs. IADLs	Bathing, dressing, toileting, eating vs. meds, meals, shopping, finances, transportation
ROS	Subjective only; pertinent positives and negatives

Self-Check ADDED

1. Which documentation is best: "Patient is anxious" or "Patient is tapping foot and asking repeatedly when the doctor will arrive"? *Answer: The second; it is descriptive.*
2. A patient arrives with a partner who answers every question and will not leave. What should the nurse do? *Answer: Find a safe way to interview the patient alone (e.g., during a private part of the exam); do not confront the partner; use a qualified interpreter if needed.*
3. A patient reports an "allergy" to codeine that causes constipation. How should the nurse interpret this? *Answer: Likely a side effect, not a true allergy; document the actual reaction.*
4. Is "managing medications" an ADL or an IADL? *Answer: IADL.*
5. Where does "crackles in the right lower lobe" belong: the ROS or the physical exam? *Answer: The physical exam (it is objective).*

Notes on Sources, Additions & Clarifications

Corrections:

1. The slide notes for the Mod 2 ROS slide and the Mod 1 health history slide both describe the ROS as subjective, but the Exam 1 study guide describes the ROS as "Subjective and Objective data collection." The slides are explicit that "objective findings belong in the physical assessment, not the ROS," so this guide follows the slides. Expect the exam to treat the ROS as subjective.

From the Exam 1 study guide (not spelled out on the slides):

- Module 2 question count (18–20) and the topic list in the blueprint box.
- The "four main areas" of the general survey (physical appearance, body structure, mobility, behavior). The slide lists eight components; the study guide names these four.
- "Three generations" for family history (the slide says a three-generation history "may be useful").
- Interpreting tachycardia, tachypnea, and fever, and the nursing actions that follow.

Added information (not in the slides):

- The box on what abnormal vital signs mean and what to do about them (written to cover the study guide question above).
- The "fever" and "bradypnea" thresholds in the vital signs table.
- The self-check questions.

Clarified:

- The common textbook expansion of PQRST (Provocation/Palliation, Region/Radiation) alongside the slide's wording.